



INPATIENT RECORD

Hospital No: 66415670	Visit No:
Name:	Age/Sex:
Doctor Name:	Specialty:
Date:	

BP Systolic: 129 mmHg	BP Diastolic: 90 mmHg	Mean Arterial Pressure-MAP: 103 mmHg
Pulse Rate: 110 bt/min	Respiration: 24 br/min	Temperature: 98.6 °F
Oxygen Delivery Device: RA	Saturation(Oxygen): 99 %	BP Systolic: 130 mmHg
BP Systolic: 150 mmHg IP Initial Assessment Score : 0 Patient Arrival Date : 24/03/2026 Patient Arrival Time : 10:30 Chief Complaints : H/O 2 EPISODES OF GTCS (ONE IN HOME AND OTHER ONE AT HOSPITAL GATE) TODAY MORNING. History of Presenting illness : 59Y/M K/C/O MENINGO-ENCEPHALITIS - CSF TB GENE XPRT POSITIVE - STARTED ON ATT WITH STEROIDS ON 12/11/2025 presented now with 2 episodes of seizures with semiology of left upper limb and lower limb tonic clonic movements with drooling and ? GTCS .GRBS - 223 Mild neck rigidity .In Post ictal confusion Bilateral plantar - Mute Allergy : NIL Past Medical History : TB meningo-encephalitis with seizures and left hemiparesis K/C/O DM SACRAL PRESSURE SORE GRADE 2 Past Surgical History : tracheostomy decannulation was done on 17/12/2025 Current medications : CAP. RIFAMPICIN 450MG 1-0-0 BEFORE FOOD TAB. ISONIAZID 300MG 1-0-0 TAB. PYRAZINAMIDE 750MG, 1½-0-0 TAB. ETHAMBUTOL 800MG 1-0-0 TAB. LEVOFLOXACIN 750MG 1-0-0 TAB. BENADON 40MG ½-0-0 TAB. DEXAMETHASONE 4MG 1-1-0 TAB. LEVIPIL 1GM 1-1-1 TAB. PAN 40MG 1-0-0 TAB. SHELICAL 500MG 1-0-0 Pulse /min : 88 Resp Rate/min : 13 Temp F : 98 BP mm Hg : 110/67 GRBS mg/dL : 232 Pallor : NO Icterus : NO Edema : NO Clubbing : NO Cyanosis : NO		



Hospital No: 66415670
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Doctor Name:

Lymphadenopathy : NO
CVS : S1S2
RS : CLEAR
PA : SOFT
CNS : DROWSSY
Others : .
Provisional Diagnosis : Breakthrough seizures, focal epilepsy
Tubercular meningo encephalitis on ATT
DIABETES MELLITUS
Care Plan : INJ LEVIPIL 1000MG IV 1-1-1
IV FLUID NS @75ML/HOUR
NEUROMONITORING
send RP-2, SGOT, SGPT
Spo2 : 98 ON RA
Diet Plan : .

Apache II scoring sheet
A.ACUTE PHYSIOLOGY SCORE : 6
B.Age : 3
C.CHRONIC HEALTH POINTS : 5
APACHEII SCORE="A(ACUTE" PHYSIOLOGY SCORE)+B(AGE POINTS)+C(CHRONIC HEALTH POINTS):14
DONE BY : DR DARU

Forms

Fall Risk Assessment Tool

Total Fall Risk Score(sum of all points per category) : 8
Remarks : 8-moderate fall risk
Age : Below 60 Years - 0
Patient care Equipment : Two Present - 2
Mobility : Requires assistance or supervision for mobility,transfer or ambulation - 2
Cognition : Lack of understanding of One's Physical and cognitive limitations- 4
Type of Assessment : Initial Assessment
Pain Assessments
Score: 2
Type of Assessment: Initial Assessment
Pain Assessment Scale: Wong Baker Scale (3 Years & Older)
Wong Baker : 2 Hurts a little bit

Additional Pain Assessment Details

Since: 24/03/2026
Type: Not Applicable



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Duration: Not Applicable
Continuous: Not Applicable
Time: Not Applicable

Associated Symptoms: None

Pain Assessments
Score: 4
Type of Assessment: Initial Assessment
Pain Assessment Scale: Behavioral Pain Scale

Facial Expression: Relaxed
Upper Limbs: Partially Bent
Compliance with ventilation: Tolerating movement
Additional Pain Assessment Details

NURSING INITIAL ASSESSMENT - NEW
Status : Entered
Score : 0
Patient Name : MR PAUL PRASANNA KUMAR E
MRN : MH017532346
Age : 59 YEARS
Gender : MALE
Patient arrival Date : 24/03/2026
Patient Arrival Time : 10:30
Accompanied by Relative : Yes
Name of Relative : MRS HELEN GLORY
Relationship with Patient : WIFE
Contact Number : 8217794825
Primary Language Spoken : TAMIL
Interpreter Needed : Yes
Room Orientation Given : No
Patient Identification Band Fixed : Yes
If Yes Type Of Band : GREEN
Height(Cm) : .
Weight(Kgs) : .
Reason Not Able To Check : NOT ABLE TO STAND



Hospital No: 66415670	Visit No:
Name:	Doctor Name:

Chief Complaints : SEIZURE DISORDER 1 EPISODES SINCE TODAY MORNING

Past History : TB meningitis with multiple seizure

Left hemiparesis

Diabetes mellitus

Vital Assessment

Temp(F) : 98.6

Pulse/min : 110

Blood Pressure(mmHg) : 120\90

Respiration/min : 24

Spo2(%) : 98

GRBS (mg/dl) : 270

Skin Integrity : INTACT

Other Parameters

EWS Score : .

GCS : e4v1m5

Pain Score : 2

Valuable handover checklist completed : Yes

Belongings : No

Dentures : No

Hearing Aid : No

Eye Glasses/Contact Lens : No

Others : NIL

Vulnerable : No

Any Special Care Given : NIL

Restraints : No

If Yes, Type Of Restraint : NIL

Any Known Allergies : UNKNOWN

Current Medications : CAP. RIFAMPICIN 450MG 1-0-0

TAB. ISONIAZID 300MG 1-0-0

TAB. PYRAZINAMIDE 750MG, 1½-0-0

TAB. ETHAMBUTOL 800MG 1-0-0

TAB. LEVOFLOXACIN 750MG 1-0-0

TAB. BENADON 40MG ½-0-0

TAB. DEXAMETHASONE 4MG 1 - 0.5 - 0

4MG 1 - 0 - 0

4MG 0.5 - 0 - 0

TAB. LEVIPIL 1GM 1-1-1

TAB. PAN 40MG 1-0-0

TAB. SHELCAL 500MG 1-0-0

Informed the Doctor : Yes

Name of the Doctor : DR. Jaychandran R

Investigations Ordered : ecg, cbc sgpt serum calcium phosphorous magnesium

Diet : NIL

Collected Old records : Yes

If Yes Document Type : 1 OLD FILE

Is the patient at risk for any of the following? If Yes, complete the risk assessment form.



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Visit No:
Doctor Name:

Fall : YES
Pressure Score : YES
DVT : YES
Aspiration : YES
Pain Assessment Score : Not Applicable
Ability to perform activities of daily living
Activity
Bathing : Dependent
Dressing : Dependent
Eating : Dependent
Walking : Dependent
Toilet Use : Dependent
Nursing needs(complete Nursing care plan based on the identified needs)
Actual needs : TO PROMOTE NORMAL HEALTH STATUS
Potential needs : TO PREVENT DVT
TO PREVENT PRESSURE ULCER
Any other needs : NIL
Assessment Done By : Iral Disha Dsouza
Rating : 2
BMI : NaN
Fall Score : 8-MODERATE FALL RISK
Pressure score : 14-MODERATE RISK
DVT Score : 8-HIGHEST
Aspiration score : 1-LOW RISK
General Physical Examination:
Pallor : NO
Icterus : NO
Edema : NO
Clubbing : NO
Cyanosis : NO
Lymphadenopathy : NO
Any Other Findings : NIL
ADULT & PAEDIATRIC - ASPIRATION RISK ASSEMENT TOOL
Status : Entered
Score : 0
CATEGORIES
MEDICAL DIAGNOSIS : Other Movement Disorders - 1

TOTAL SCORE : 1
COMMENTS : 1-low risk
Adult Pressure Ulcer Braden Scale
Status : Entered
SENSORY PERCEPTION : 2
2 - Very Limited : Y
MOISTURE : 3
3 - Occasionally Moist : Y
ACTIVITY : 1



Hospital No: 66415670
Name:

Visit No:
Doctor Name:

1 - Bed Bound : Y
MOBILITY : 3
3 - Slightly Limited : Y
NUTRITION : 3
3 - Adequate : Y
FRICTION AND SHEAR : 2
2 - Potential Problem : Y
TOTAL SCORE : 14-moderate risk
Status : Entered
Questionnaires
Status : Entered
Date : 24/03/2026
Patient's Identification No. : MH017532346
Sex : Male
A - Each Risk Factor Represents 1 Point : Age 41 - 60 years,Medical patient currently at bed rest,Other risk factor
Total Tickmarks : 3
Total Score A : 3
D - Each Risk Factor Represents 5 Points : Acute spinal cord injury (paralysis) (< 1 month)
Total Tickmarks : 1
Total Score D : 5
Total Score (A+B+C+D+E) = : 8
Risk Assessment : : Highest
3 - 4 High : 8-highest
Patient Arrival Date and Time
Status : Entered
Score : 0
Patient Arrival
Date : 24/03/2026
Time : 10:30

Forms

Add Nursing Care Plan
Status : Entered
Nursing Care Plan discussed with Patient : No
Nursing Care Plan discussed with Family : No
Patient Measurable Goal for the day : to prevent risk for dvtNursing Diagnosis : Risk for DVT
Nursing Interventions : >Thromboprophylaxis

>Pulmonary embolism

>Prevent from deep vein thrombosis

>Assess the condition of the patient
>Encouraged regular exercise, limb elevation and mobility
>Applied compression stockings
>Apply mechanical and pharmacological DVT measures
>Monitored closely for signs of DVT



Hospital No: 66415670
Name:

Visit No:
Doctor Name:

>Educated patient and family on DVT prevention strategies

Evaluation : Continue

Add Nursing Care Plan

Status : Entered

Nursing Care Plan discussed with Patient : No

Nursing Care Plan discussed with Family : No

Patient Measurable Goal for the day : to prevent risk for fallNursing Diagnosis : Risk for fall

Nursing Interventions : >Safety measures and motility assistance

>Prevent from injury

>Prevent the patient from fall

>Assess the condition of the patient

>Implemented safety measures like : bed side rails raised up and call bell kept within reach.

>Provided assistance with mobility

>Reviewed the medications that may increase fall risk.

>Educated the patient and family on fall prevention strategies.

Evaluation : Continue

Add Nursing Care Plan

Status : Entered

Nursing Care Plan discussed with Patient : No

Nursing Care Plan discussed with Family : No

Patient Measurable Goal for the day : to prevent risk of pressure ulcerNursing Diagnosis : Risk of pressure ulcer/

Injury can cause by lack of mobilization due to pain and altered nutritional diet

Nursing Interventions : Monitor skin integrity every 2nd hourly.

b)Position the patient to stay off a the ulcer

c)Re-position the patient every 2nd hourly

d)Apply pressure reducing devices (Air mattress)

e)Avoid massage over the bony prominent

F)Educate wheel chair bounded patient for reposition

Evaluation : Continue

Forms

Add Nursing Care Plan

Status : Entered

Nursing Care Plan discussed with Family : Yes

Patient Measurable Goal for the day : to prevent dvtNursing Diagnosis : Risk for DVT

Nursing Interventions : >Thromboprophylaxis

>Pulmonary embolism

>Prevent from deep vein thrombosis

>Assess the condition of the patient

>Encouraged regular exercise, limb elevation and mobility

>Applied compression stockings



Hospital No: 66415670
Name:

Visit No:
Doctor Name:

- >Apply mechanical and pharmacological DVT measures
- >Monitored closely for signs of DVT

>Educated patient and family on DVT prevention strategies

Evaluation : Continue

Add Nursing Care Plan

Status : Entered

Nursing Care Plan discussed with Family : Yes

Patient Measurable Goal for the day : to prevent pressure ulcerNursing Diagnosis : Risk of pressure ulcer/Injury can cause by lack of mobilization due to pain and altered nutritional diet

Nursing Interventions : >Regular skin assessment

>Pressure ulcer

>Prevent the patient from pressure ulcer and maintain normal skin integrity

>Assessed the skin status of the patient

Positioning done 2nd hourly

Back care given

Encouraged mobility and activity

Provided adequate nutrition

>Educated patient and family on pressure ulcer preventions

Evaluation : Continue

Add Nursing Care Plan

Status : Entered

Nursing Care Plan discussed with Family : Yes

Patient Measurable Goal for the day : to prevent from risk for fallNursing Diagnosis : Risk for fall

Nursing Interventions : >Safety measures and motility assistance

>Prevent from injury

>Prevent the patient from fall

>Assess the condition of the patient

>Implemented safety measures like : bed side rails raised up and call bell kept within reach.

>Provided assistance with mobility

>Reviewed the medications that may increase fall risk.

>Educated the patient and family on fall prevention strategies.

Evaluation : Continue

Alerts



Hospital No: 66415670
Name:

Visit No:
Doctor Name:

- Onset: 24/03/2026
EJV 16 G DAY 1

Forms

CAUTI Maintenance Bundle Checklist

Status : Entered

Score : 0

Bundle Criteria : Morning Shift

Day : D4

Regular documentation for assessment for need of catheter : YES

The Urinary catheter has been continuously connected : YES

Date & time meatal hygiene done : YES

Drainage bag emptied into a clean : YES

Container : YES

Hand hygiene performed for patient contact : YES

Catheter secured : YES

Keep the drianage bag above the floor, but below the bladder level to prevent reflux / contamination : YES

REMARKS : FOLYES DAY 1

RESTRAIN MONITERING CHART

Status : Entered

CONSENT : YES

Date : 24/03/2026

Time : 10:00

DISCOLOURATION : No

EDEMA : No

SKIN PEEL : No

ANY OTHER : nil

RELEASE EVERY SECOND HOURLY : Yes

REMARKS : nil

Date : 24/03/2026

Time : 12:00

DISCOLOURATION : No

EDEMA : No

SKIN PEEL : No

RELEASE EVERY SECOND HOURLY : Yes

REMARKS : nil

Date : 24/03/2026

Time : 16:00

DISCOLOURATION : No

EDEMA : No

RELEASE EVERY SECOND HOURLY : Yes

Date : 24/03/2026

Time : 16:00

DISCOLOURATION : No

EDEMA : No

SKIN PEEL : No

RELEASE EVERY SECOND HOURLY : Yes

Date : 24/03/2026



Hospital No: 66415670	Visit No:
Name:	Doctor Name:

Time : 18:00
DISCOLOURATION : No
EDEMA : No
SKIN PEEL : No
RELEASE EVERY SECOND HOURLY : Yes
Date : 24/03/2026
Time : 20:00
DISCOLOURATION : No
EDEMA : No
SKIN PEEL : No
RELEASE EVERY SECOND HOURLY : Yes
Date : 24/03/2026
Time : 22:00
EDEMA : No
SKIN PEEL : No
RELEASE EVERY SECOND HOURLY : Yes
Date : 25/03/2026
Time : 00:00
DISCOLOURATION : No
EDEMA : No
SKIN PEEL : No
RELEASE EVERY SECOND HOURLY : Yes
Date : 25/03/2026
Time : 02:00
DISCOLOURATION : No
EDEMA : No
SKIN PEEL : No
RELEASE EVERY SECOND HOURLY : Yes
Date : 25/03/2026
Time : 04:00
DISCOLOURATION : No
EDEMA : No
SKIN PEEL : No
RELEASE EVERY SECOND HOURLY : Yes
Date : 25/03/2026
Time : 06:00
DISCOLOURATION : No
EDEMA : No
SKIN PEEL : No
RELEASE EVERY SECOND HOURLY : Yes
Date : 24/03/2026
Time : 10:00
DISCOLOURATION : No
EDEMA : No
SKIN PEEL : No
ANY OTHER : nil
RELEASE EVERY SECOND HOURLY : Yes



Hospital No: 66415670
Name:

Visit No:
Doctor Name:

REMARKS : nil
Date : 24/03/2026
Time : 12:00
DISCOLOURATION : No
EDEMA : No
SKIN PEEL : No
RELEASE EVERY SECOND HOURLY : Yes
REMARKS : nil
Date : 24/03/2026
Time : 16:00
DISCOLOURATION : No
EDEMA : No
RELEASE EVERY SECOND HOURLY : Yes
Date : 24/03/2026
Time : 16:00
DISCOLOURATION : No
EDEMA : No
SKIN PEEL : No
RELEASE EVERY SECOND HOURLY : Yes
Date : 24/03/2026
Time : 18:00
DISCOLOURATION : No
EDEMA : No
SKIN PEEL : No
RELEASE EVERY SECOND HOURLY : Yes
Date : 24/03/2026
Time : 20:00
DISCOLOURATION : No
EDEMA : No
SKIN PEEL : No
RELEASE EVERY SECOND HOURLY : Yes
Date : 24/03/2026
Time : 22:00
EDEMA : No
SKIN PEEL : No
RELEASE EVERY SECOND HOURLY : Yes
Date : 25/03/2026
Time : 00:00
DISCOLOURATION : No
EDEMA : No
SKIN PEEL : No
RELEASE EVERY SECOND HOURLY : Yes
Date : 25/03/2026
Time : 02:00
DISCOLOURATION : No
EDEMA : No
SKIN PEEL : No



Hospital No: 66415670
Name:

Visit No:
Doctor Name:

RELEASE EVERY SECOND HOURLY : Yes

Date : 25/03/2026

Time : 04:00

DISCOLOURATION : No

EDEMA : No

SKIN PEEL : No

RELEASE EVERY SECOND HOURLY : Yes

Date : 25/03/2026

Time : 06:00

DISCOLOURATION : No

EDEMA : No

SKIN PEEL : No

RELEASE EVERY SECOND HOURLY : Yes

Patient Acuity Tool

Stable patient 1 to 11

Moderate Risk Patient 12 to 22

Complex Patient 22 to 33

High Risk Patient >34

Total Score :28

Forms

Nurses Endorsment Checklist

Status : Entered

Shift : Morning to Evening

Situation : ER,NON MLC,Shift Handover

Comment / Chief Complaints / Diagnosis : encephalitis

Background

Allergies : No

Remarks : UNKNOWN

Diet : Yes

Remarks : soft diet

Previous Medications : No

Remarks : NIL

Old reports added to TRAK : No

Remarks : NIL

Consents : No

Remarks : NIL

Cross Consultation : No

Remarks : NIL

Procedures performed before transfer : No

Remarks : NIL

Vulnerable : Yes

Remarks : altered sensorium

Past Medical / Surgical History : TYPE 2

DM

Level of Consciousness : Confused



Hospital No: 66415670	Visit No:
Name:	Doctor Name:

GCS : e4v3m5
 Vitals -TPR & BP : is stable : Yes
 Pain Score : 2\10
 On MET/PET List : No
 Risk For FALL : Yes
 Remarks : 8-moderate fall risk
 Risk For Pressure Ulcer : Yes
 Remarks : 14-moderate risk
 Risk for Aspiration : Yes
 Remarks : 1-low risk
 Existence of Pressure Ulcer : No
 Remarks : INTACT
 O2 Administration : No
 Remarks : room air
 SPO2(Latest reading in the shift) : 99
 DIET : NGTube
 Diet-Specify the type : Diabetic
 On tube feeding : Yes
 If yes, Specify the type : Nasogastric
 IV Fluids : Yes
 Remarks : IVF NS 50 ML\HR
 Blood/Blood products : No
 Total Parenteral Nutrition : No
 Pre Antibiotic Culture : No
 Invasive Lines : Yes
 Remarks : EJV 20 G DAY 1
 Self Void : No
 Foley's : Yes
 Remarks : 14F FOLYES DAY 1
 Drain : No
 Bowel Opened : No
 Is the patient posted for any Surgery / Procedure : No
 b)If Yes, is the financial clearance obtained (As applicable) : a)If Yes, Refer the pre-operative checklist
 Dressing Present : No
 High Risk medications : No
 Is Patient on Physiotherapy : No
 Ambulation : Bed Bound
 Investigations Sent : No
 Reports Collected : No
 Care Plan : Yes
 Remarks : fall ,DVT, NUTRITION
 Special Needs : Yes
 Remarks : neuromonitoring
 Patient & Family Education : Yes
 Remarks : hand hygiene
 Transfer : No
Medications: No



Hospital No: 66415670

Visit No:

Name:

Doctor Name:

Planned Procedure : No
Approved follow-up : No
Transfusion : No
Patient belongings with health care providers : No
Pending Reports : No
Critical Value : No
Pharmacy / Consumables : Charged
Remarks : pharmacy
Discharge Plan : No
Comments : GCS MONITORING
Hand over by : Iral Disha Dsouza
Handed over to : Jasmine K J
Team Lead Hand over By : Sreeja
Team Lead Hand over To : Joydip Dey
Status : Entered

Forms

CAUTI Maintenance Bundle Checklist

Score : 0

Bundle Criteria : Night Shift

Day : D3

Regular documentation for assessment for need of catheter : YES

The Urinary catheter has been continuously connected : YES

Date & time meatal hygiene done : YES

Drainage bag emptied into a clean : YES

Container : YES

Hand hygiene performed for patient contact : YES

Catheter secured : YES

Keep the drianage bag above the floor, but below the bladder level to prevent reflux / contamination : YES

Fall Risk Assessment Tool

Total Fall Risk Score(sum of all points per category) : 6

Age : Below 60 Years - 0

Elimination,Bowel and Urine : Incontinence- 2

Patient care Equipment : Two Present - 2

Mobility : Requires assistance or supervision for mobility,transfer or ambulation - 2

Type of Assessment : Shift Assessment

Skin Examination

Pallor : No

Cyanosis : No

Skin turgor : Normal turgor

Jaundice : No

Erythema : No

Petechiae : No

Purpura : No



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Oedema : No
 Rash : No
 Skin Lesions : No
 Surgical scars : No
 Wounds : Yes
 Location : pressure ulcer present in sacral region
 Burns : No
 Other : ,(heled) 2nd degree bed sore
 Hair : Normal

Nails : Normal

Pain Assessments
 Score: 2
 Type of Assessment: Shift Assessment
 Pain Assessment Scale: Numeric Pain Scale (0-10 Scale)
 Numeric Pain Scale: 2

Additional Pain Assessment Details

Since: 24/03/2026
 Type: Dull
 Duration: Not Applicable
 Continuous: Not Applicable
 Time: Not Applicable
 Onset: Sudden

Associated Symptoms: None

Patient Acuity Tool
 Stable patient 1 to 11
 Moderate Risk Patient 12 to 22
 Complex Patient 22 to 33
 High Risk Patient >34

Total Score :38
ADULT & PAEDIATRIC - ASPIRATION RISK ASSEMENT TOOL
 Score : 0
 CATEGORIES



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TOTAL SCORE : 2

Adult Pressure Ulcer Braden Scale

SENSORY PERCEPTION : 2

2 - Very Limited : Y

MOISTURE : 3

3 - Occasionally Moist : Y

ACTIVITY : 1

1 - Bed Bound : Y

MOBILITY : 2

2 - Very Limited : Y

NUTRITION : 3

3 - Adequate : Y

FRICITION AND SHEAR : 2

2 - Potential Problem : Y

TOTAL SCORE : 13

Questionnaires

Date : 24/03/2026

Patient's Identification No. : MH017532346

Sex : Male

A - Each Risk Factor Represents 1 Point : Age 41 - 60 years,Medical patient currently at bed rest,Other risk factor

Total Tickmarks : 3

Total Score A : 3

Total Score (A+B+C+D+E) = : 3

Risk Assessment : : High

Forms

Nurses Endorsment Checklist

Shift : Evening to Night

Situation : Elective,NON MLC,Shift Handover

Comment / Chief Complaints / Diagnosis : encephalitis

Background

Allergies : No

Remarks : UNKNOWN

Diet : Yes

Remarks : RT Feed

Previous Medications : No

Remarks : NIL

Old reports added to TRAK : No

Remarks : NIL

Consents : Yes

Remarks : restrain consent

Cross Consultation : No

Remarks : NIL

Procedures performed before transfer : No

Remarks : NIL

Vulnerable : Yes

Remarks : altered sensorium

Past Medical / Surgical History : TYPE 2 DM



Hospital No: 66415670

Visit No:

Name:

Doctor Name:

Level of Consciousness : Confused
GCS : e3m5v3
Vitals -TPR & BP : is stable : Yes
Pain Score : 2/10
On MET/PET List : No
Risk For FALL : Yes
Remarks : 12 moderate risk
Risk For Pressure Ulcer : Yes
Remarks : 13 moderate risk
Risk for Aspiration : Yes
Remarks : 2 Low risk
Existence of Pressure Ulcer : No
Remarks : skin is intact
O2 Administration : No
SPO2(Latest reading in the shift) : 96
DIET : NGTube
Diet-Specify the type : Diabetic
On tube feeding : Yes
If yes, Specify the type : Nasogastric
IV Fluids : Yes
Remarks : NS 75 ml/hr
Blood/Blood products : No
Total Parenteral Nutrition : No
Pre Antibiotic Culture : No
Invasive Lines : Yes
Remarks : 16 G EJV -DAY 1
-20 G venflon -day 1
Self Void : No
Foley's : Yes
Remarks : 16 fr foleys catheter
Drain : No
Bowel Opened : No
Is the patient posted for any Surgery / Procedure : No
b)If Yes, is the financial clearance obtained (As applicable) : a)If Yes, Refer the pre-operative checklist
Dressing Present : No
High Risk medications : No
Is Patient on Physiotherapy : No
Ambulation : Bed Bound
Investigations Sent : No
Reports Collected : No
Care Plan : No
Inotropes : No
Ventilator : No
Dialysis : No
Special Needs : Yes
Remarks : Foleys care
Patient & Family Education : Yes



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Doctor Name:

Remarks : Hand Hygiene
Transfer : No
Medications: No
Planned Procedure : No
Approved follow-up : No
Transfusion : No
Patient belongings with health care providers : No
Pending Reports : No
Critical Value : No
Pharmacy / Consumables : Replaced
Discharge Plan : No
Hand over by : Jasmine K J
Handed over to : PARVAIZ AHMAD MIR
Team Lead Hand over By : Joydip Dey
Team Lead Hand over To : Archana NC
Add Nursing Care Plan
Status : Entered
Nursing Care Plan discussed with Family : Yes
Patient Measurable Goal for the day : to maintain the hydration status
Nursing Diagnosis : Risk for fluid volume deficit
Nursing Interventions : -Monitor the skin integrity
-Administer iv fluids as per doctors order
-Monitor intake and output
Evaluation : Continue
Add Nursing Care Plan
Status : Entered
Nursing Care Plan discussed with Patient : No
Nursing Care Plan discussed with Family : Yes
Patient Measurable Goal for the day : to prevent the fall
Nursing Diagnosis : Risk for fall
Nursing Interventions : a) Orient to surroundings on admission.
b) Provide education on how to use call bell and observe the patient demonstration.
c) Provide appropriate safety measures like sitter, display vulnerable patient alert
Evaluation : Continue
Add Nursing Care Plan
Status : Entered
Nursing Care Plan discussed with Patient : No
Nursing Care Plan discussed with Family : Yes
Patient Measurable Goal for the day : to reduce the pain
Nursing Diagnosis : Acute pain ./ or post-operative pain
Nursing Interventions : a) Monitor characteristics of pain (Location, severity using
Appropriate pain scale), frequency, precipitating factors and relief factors.
B) Eliminate factors that precipitate pain.
C) Other analgesic 6th/8th hourly if necessary in accordance with physician
Evaluation : Continue
Patient/Family Health Education
Status : Entered
Date : 24/03/2026
Time : 16:06



Hospital No: 66415670
Name:

Visit No:
Doctor Name:

Assessment Type : Other1

Educational Needs : Hand hygiene
Given Education : Hand hygiene
Teaching Method : One to One
Teaching Tool : Verbal Instructions
Evaluation : Verbalize basic concept with assistance
Child Attends School : Not Applicable
Does the child has any problem at school : Not Applicable

Forms

CAUTI Maintenance Bundle Checklist
Status : Entered
Score : 0
Bundle Criteria : Night Shift
Day : D4
Regular documentation for assessment for need of catheter : YES
The Urinary catheter has been continuously connected : YES
Date & time meatal hygiene done : YES
Drainage bag emptied into a clean : YES
Container : YES
Hand hygiene performed for patient contact : YES
Catheter secured : YES
Keep the drianage bag above the floor, but below the bladder level to prevent reflux / contamination : YES
Fall Risk Assessment Tool

Total Fall Risk Score(sum of all points per category) : 7
Remarks : 7 Moderate Fall Risk
Age : Below 60 Years - 0
Elimination,Bowel and Urine : Urgency or frequency - 2
Patient care Equipment : Two Present - 2
Mobility : Requires assisstance or supervision for mobility,transfer or ambulation - 2
Cognition : Altered awareness of immediate physical environment- 1
Type of Assessment : Shift Assessment
Skin Examination
Pallor : No
Cyanosis : No
Jaundice : No
Erythema : No
Petechiae : No
Oedema : No
Rash : No
Skin Lesions : No
Surgical scars : No
Wounds : Yes



Hospital No: 66415670
Name:

Visit No:
Doctor Name:

Location : pressure ulcer present in sacral region
Burns : No
Other : ,(heled) 2nd degree bed sore
Hair : Normal

Nails : Normal

Pain Assessments
Score: 2
Type of Assessment: Shift Assessment
Pain Assessment Scale: Numeric Pain Scale (0-10 Scale)
Numeric Pain Scale: 2

Additional Pain Assessment Details

Patient Acuity Tool
Stable patient 1 to 11
Moderate Risk Patient 12 to 22
Complex Patient 22 to 33
High Risk Patient >34

Total Score :23
ADULT & PAEDIATRIC - ASPIRATION RISK ASSEMENT TOOL
Status : Entered
Score : 0
CATEGORIES

TOTAL SCORE : 4
COMMENTS : 4 Moderate Risk
Adult Pressure Ulcer Braden Scale
Status : Entered
SENSORY PERCEPTION : 3
3 - Slightly Limited : Y
MOISTURE : 3
3 - Occasionally Moist : Y
ACTIVITY : 1
1 - Bed Bound : Y



Hospital No: 66415670
Name:

Visit No:
Doctor Name:

MOBILITY : 3
3 - Slightly Limited : Y
NUTRITION : 2
2 - Probably Inadequate : Y
FRICTION AND SHEAR : 1
1 - Problem : Y
TOTAL SCORE : 13 Moderate Risk
Status : Entered
Questionnaires
Status : Entered
Date : 24/03/2026
Sex : Male
A - Each Risk Factor Represents 1 Point : Age 41 - 60 years, Medical patient currently at bed rest, Other risk factor
Total Tickmarks : 3
Total Score A : 3
Total Score (A+B+C+D+E) = : 3
Risk Assessment : : High
3 - 4 High : 3 High risk

Forms

Add Nursing Care Plan
Status : Entered
Nursing Care Plan discussed with Family : Yes
Patient Measurable Goal for the day : to prevent dvt
Nursing Diagnosis : Risk for DVT
Nursing Interventions : provide passive exercises

provide elevation of limbs

provide flowtron pump
Evaluation : Continue
Add Nursing Care Plan
Status : Authorised
Nursing Care Plan discussed with Family : Yes
Patient Measurable Goal for the day : to relieve pain
Nursing Diagnosis : Acute pain ./ or post-operative pain
Nursing Interventions : asses the level of pain

provide analgesics as per order

provide diversional therapy
Add Nursing Care Plan
Status : Entered
Nursing Care Plan discussed with Family : Yes
Patient Measurable Goal for the day : to prevent pressure ulcer
Nursing Diagnosis : Risk of pressure ulcer/Injury can cause by lack of mobilization due to pain and altered nutritional diet
Nursing Interventions : provide back care

provide alpha bed



Hospital No: 66415670
Name:

Visit No:
Doctor Name:

change position 2nd hourly
Evaluation : Continue
Patient/Family Health Education
Status : Entered
Date: : 24/03/2026
Time: : 20:19
Assessment Type : Other1

Educational Needs : Risk,Hand hygiene,Pressure ulcer precaution
Given Education : Hand hygiene
Teaching Method : One to One
Teaching Tool : Verbal Instructions
Evaluation : Other1
Child Attends School : Yes
Does the child has any problem at school : No
Literacy Skills : Yes

Progress Note :
No further seizures
EEG - right hemispheric periodic discharges and slowing
No fever
persists to be in alteed sensorium
Left hemiparesis -likely postictal
Plan:
MRI brain screening tomorrow if no improvement
Continue medications as per chart

Progress Note :
C/S/B ICU TEAM

Breakthrough seizures - Focal epilepsy
TB meningo-encephalitis with seizures and left hemiparesis
DM
S/P Tracheostomy and decannulation on 17/12/25

Patient reviewed
Patient is conscious, slurred speech +
EEG on going

Vitals:
HR - 105bpm
BP- 160/90 mm Hg
SPO2- 96% on RA
RR - 20cpm

Advice:
- Monitor vitals



Hospital No: 66415670
Name:

Visit No:
Doctor Name:

- Treatment as per chart
- Watch for seizures
- Trace EEG report
- MRI tomorrow
- Inform SOS

Forms

Nurses Endorsment Checklist
Status : Entered
Shift : Night to Morning
Situation : Elective,NON MLC,Shift Handover
Comment / Chief Complaints / Diagnosis : encephalitis
Background
Allergies : No
Remarks : UNKNOWN
Diet : Yes
Remarks : RT Feed
Previous Medications : No
Remarks : NIL
Old reports added to TRAK : No
Remarks : NIL
Consents : Yes
Remarks : restrain consent
Cross Consultation : No
Remarks : NIL
Procedures performed before transfer : No
Remarks : NIL
Vulnerable : Yes
Remarks : altered sensorium
Past Medical / Surgical History : TYPE 2 DM
Intubation : no
Ventilation Status : on room air spo2 96%
ET Size and Lip Level : no
Tracheostomy : no
ICD : no
Pig Tail : no
T Piece : no
Chest Physio : no
Treated with Inotropes : no
Level of Consciousness : Oriented
GCS : e3m5v3
Vitals -TPR & BP : is stable : Yes
Pain Score : 2
On MET/PET List : No
Risk For FALL : Yes
Remarks : 7 Moderate Fall Risk
Risk For Pressure Ulcer : Yes
Remarks : 13 Moderate Risk



Hospital No: 66415670
Name:

Visit No:
Doctor Name:

Risk for Aspiration : Yes
Remarks : : 4 Moderate Risk
Existence of Pressure Ulcer : No
O2 Administration : No
SPO2(Latest reading in the shift) : 96
DIET : NGTube
Diet-Specify the type : Diabetic
On tube feeding : Yes
If yes, Specify the type : Nasogastric
IV Fluids : Yes
Remarks : ns@75ml/hr
Blood/Blood products : No
Total Parenteral Nutrition : No
Pre Antibiotic Culture : No
Invasive Lines : Yes
Remarks : 20g venflon and 16 g ejv
Self Void : No
Foley's : Yes
Remarks : 14fr foleys outside
Drain : No
Bowel Opened : Yes
Remarks : 1 time
b)If Yes, is the financial clearance obtained (As applicable) : a)If Yes, Refer the pre-operative checklist
Dressing Present : No
High Risk medications : No
Is Patient on Physiotherapy : No
Ambulation : Bed Bound
Investigations Sent : Yes
Remarks : electrolytes
Reports Collected : Yes
Care Plan : Yes
Remarks : dvt pain pressure ulcer
Inotropes : No
Ventilator : No
Dialysis : No
Special Needs : Yes
Remarks : asses neurological status
Patient & Family Education : Yes
Transfer : No
Medications: Yes
Planned Procedure : No
Approved follow-up : Yes
Transfusion : No
Patient belongings with health care providers : No
Pending Reports : No
Critical Value : No
Pharmacy / Consumables : Charged



Hospital No: 66415670
Name:

Visit No:
Doctor Name:

Remarks : Consumables
Discharge Plan : No
Hand over by : PARVAIZ AHMAD MIR
Handed over to : Mr Pushpraj Manoj Khandagale
Team Lead Hand over By : Archana NC
Team Lead Hand over To : Joydip Dey
Status : Entered

Forms

CAUTI Maintenance Bundle Checklist
Status : Entered
Score : 0
Bundle Criteria : Morning Shift
Day : D1
Regular documentation for assessment for need of catheter : YES
The Urinary catheter has been continuously connected : YES
Date & time meatal hygiene done : YES
Drainage bag emptied into a clean : YES
Container : YES
Hand hygiene performed for patient contact : YES
Catheter secured : YES
Keep the drianage bag above the floor, but below the bladder level to prevent reflux / contamination : YES
Fall Risk Assessment Tool

Total Fall Risk Score(sum of all points per category) : 13
Remarks : moderate risk
Age : Below 60 Years - 0
Elimination,Bowel and Urine : Urgency or frequency - 2

Medications:

ON 1 HIGH FALL RISK DRUG - 3
Patient care Equipment : Two Present - 2
Mobility : Requires assistance or supervision for mobility,transfer or ambulation - 2
Cognition : Lack of understanding of One's Physical and cognitive limitations- 4
Type of Assessment : Shift Assessment
Skin Examination
Pallor : No
Cyanosis : No
Jaundice : No
Erythema : No
Petechiae : No
Oedema : No
Rash : No
Skin Lesions : No
Surgical scars : No



Hospital No: 66415670

Visit No:

Name:

Doctor Name:

Wounds : Yes
Location : pressure ulcer present in sacral region
Burns : No
Other : ,(heled) 2nd degree bed sore
Hair : Normal

Nails : Normal

Pain Assessments
Score: 2
Type of Assessment: Shift Assessment
Pain Assessment Scale: Numeric Pain Scale (0-10 Scale)
Numeric Pain Scale: 2

Additional Pain Assessment Details

Since: 25/03/2026
Type: Not Applicable
Duration: Not Applicable
Continuous: Not Applicable
Time: Not Applicable

Patient Acuity Tool
Stable patient 1 to 11
Moderate Risk Patient 12 to 22
Complex Patient 22 to 33
High Risk Patient >34

Total Score :12
ADULT & PAEDIATRIC - ASPIRATION RISK ASSEMENT TOOL

Status : Entered

Score : 0

CATEGORIES

MEDICAL DIAGNOSIS : Seizure Disorder -
1

EATING HABILTS : Low Level of alertness that impairs complete participation in oral intake - 1,Needs assistance
to eat - 1,On Tube feeding - 3

TOTAL SCORE : 6



Hospital No: 66415670
Name:

Visit No:
Doctor Name:

COMMENTS : moderate risk
Adult Pressure Ulcer Braden Scale
Status : Entered
SENSORY PERCEPTION : 3
3 - Slightly Limited : Y
MOISTURE : 4
4 - Rarely Moist : Y
ACTIVITY : 1
1 - Bed Bound : Y
MOBILITY : 2
2 - Very Limited : Y
NUTRITION : 3
3 - Adequate : Y
FRICTION AND SHEAR : 2
2 - Potential Problem : Y
TOTAL SCORE : 15 mild risk
Status : Entered
Questionnaires
Status : Entered
Date : 25/03/2026
Sex : Male
A - Each Risk Factor Represents 1 Point : Age 41 - 60 years,Medical patient currently at bed rest,Other risk factor
Total Tickmarks : 3
Total Score A : 3
Total Score (A+B+C+D+E) = : 3
Risk Assessment : : High
3 - 4 High : 3 high risk

Forms

Nurses Endorsment Checklist
Status : Entered
Shift : Morning to Evening
Situation : Elective,NON MLC,Shift Handover
Comment / Chief Complaints / Diagnosis : encephalitis
Background
Allergies : No
Remarks : UNKNOWN
Diet : Yes
Remarks : RT Feed
Previous Medications : No
Remarks : NIL
Old reports added to TRAK : No
Remarks : NIL
Consents : Yes
Remarks : restrain consent
Cross Consultation : No
Remarks : NIL
Procedures performed before transfer : No



Hospital No: 66415670

Visit No:

Name:

Doctor Name:

Remarks : NIL
 Vulnerable : Yes
 Remarks : altered sensorium
 Past Medical / Surgical History : TYPE 2 DM
 Level of Consciousness : Confused
 GCS : e3m6v3
 Vitals -TPR & BP : is stable : Yes
 Pain Score : 2
 On MET/PET List : No
 Risk For FALL : Yes
 Remarks : 13 moderate risk
 Risk For Pressure Ulcer : Yes
 Remarks : 15 mild risk
 Risk for Aspiration : Yes
 Remarks : 6 moderate risk
 Existence of Pressure Ulcer : No
 O2 Administration : No
 SPO2(Latest reading in the shift) : 97
 DIET : NGTube
 Diet-Specify the type : Diabetic
 On tube feeding : Yes
 If yes, Specify the type : Nasogastric
 IV Fluids : Yes
 Remarks : 75ml/hr
 Blood/Blood products : No
 Total Parenteral Nutrition : No
 Pre Antibiotic Culture : No
 Invasive Lines : Yes
 Remarks : 16g ejv and 18g venflon present
 Self Void : No
 Foley's : No
 Drain : No
 Bowel Opened : No
 Is the patient posted for any Surgery / Procedure : No
 b)If Yes, is the financial clearance obtained (As applicable) : a)If Yes, Refer the pre-operative checklist
 Dressing Present : No
 High Risk medications : No
 Is Patient on Physiotherapy : Yes
 Ambulation : Bed Bound
 Investigations Sent : Yes
 Reports Collected : Yes
 Care Plan : Yes
 Remarks : pressure ulcer, fall, dvt
 Inotropes : No
 Ventilator : No
 Dialysis : No
 Special Needs : Yes



Hospital No: 66415670

Visit No:

Name:

Doctor Name:

Patient & Family Education : Yes

Transfer : No

Medications: No

Planned Procedure : No

Approved follow-up : No

Transfusion : No

Patient belongings with health care providers : No

Pending Reports : No

Critical Value : No

Pharmacy / Consumables : Replaced

Remarks : Consumables

Discharge Plan : No

Comments : neuromonitoring

Hand over by : Mr Pushpraj Manoj Khandagale

Handed over to : Kavita Naik

Team Lead Hand over By : Joydip Dey

Team Lead Hand over To : Sreeja

Status : Entered

Add Nursing Care Plan

Status : Entered

Nursing Care Plan discussed with Patient : No

Nursing Care Plan discussed with Family : Yes

Patient Measurable Goal for the day : to prevent fall Nursing Diagnosis : Risk for fall

Nursing Interventions : 1.Answer calls promptly.

2.Place a fall risk armband on the patient.

3.Place personal items within close reach.

4.Maintain a toileting schedule throughout the day.

5.Encourage wearing sturdy shoes with non-skid soles.

Educate the patient and family on fall risks and prevention measures.

Ensure call bell is within reach.

Use non-skid socks or footwear.

Keep the patient's environment free of clutter.

Assess the patient's fall risk score.

Use bed and chair alarms as needed.

Evaluation : Continue

Add Nursing Care Plan

Status : Entered

Nursing Care Plan discussed with Patient : No

Nursing Care Plan discussed with Family : Yes

Patient Measurable Goal for the day : to prevent risk for dvt Nursing Diagnosis : Risk for DVT

Nursing Interventions : Complete bed rest to avoid dislodgment of blood clots

Elevate both or affected legs

Avoid crossing legs while turning patient after every 2 hours

Variety of motion exercises to the unaffected leg

Reduce swelling by applying warm compresses

Constant monitoring of the vital signs for every 4 to 6 hours

Thorough assessment of the patient's history and symptoms



Hospital No: 66415670
Name:

Visit No:
Doctor Name:

Administering anticoagulant medications
Evaluation : Continue
Add Nursing Care Plan
Status : Entered
Nursing Care Plan discussed with Patient : No
Nursing Care Plan discussed with Family : Yes
Patient Measurable Goal for the day : to prevent risk for pressure ulcer Nursing Diagnosis : Risk of pressure ulcer/
Injury can cause by lack of mobilization due to pain and altered nutritional diet
Nursing Interventions : 1.Implement turning schedules.
2.Use pillows to protect bone prominences.
3.Maintain proper hygiene care.
4.Ensure adequate nutrition and fluid intake.
5.Assess and stage pressure injuries accurately.
6.Implement effective pressure relief and redistribution strategies.
Optimize wound care and promote healing.
Manage pain and discomfort associated with pressure injuries.
Evaluation : Continue
Patient/Family Health Education
Status : Entered
Date: : 25/03/2026
Time: : 08:16

Educational Needs : Disease process,Pesonal hygiene and practitioner hand wash,Hand hygiene,Pressure ulcer
precaution
Teaching Method : One to One
Evaluation : Verbalize basic concept without assistance,Applies knowledge/skills
Child Attends School : No
Does the child has any problem at school : No
Literacy Skills : Yes

Progress Note :
C/S/B ICU TEAM

Breakthrough seizures - Focal epilepsy
TB meningo-encephalitis with seizures and left hemiparesis
DM
S/P Tracheostomy and decannulation on 17/12/25

PATIENT DROWSY, OBEYS COMMANDS
ON ROOM AIR
NA+130/K+3.7
MRI BRAIN DONE

VITALS
BP-130/80
PR-88
SPO2-97%



Hospital No: 66415670	Visit No:
Name:	Doctor Name:

PLAN
TRACE MRI BRAIN REPORT
WATCH FOR SEIZURES
MONITOR VITALS

Progress Note :
Had one more seizure today morning
EEG done yesterday showed right hemispheric PLEDs
Afebrile
Vitals - stable
MRI brain repeated now - no acute lesions, FLAIR hyperintensities right temporal, multiple areas of gliosis

Plan:
Add Lacosamide
PT/INR stat
Lumbar puncture to be repeated

Forms

CAUTI Maintenance Bundle Checklist
Status : Entered
Score : 0
Bundle Criteria : Evening Shift
Day : D5
Regular documentation for assessment for need of catheter : YES
The Urinary catheter has been continuously connected : YES
Date & time meatal hygiene done : YES
Remarks : 25/3/2026@6AM
Drainage bag emptied into a clean : YES
Container : YES
Hand hygiene performed for patient contact : YES
Catheter secured : YES
Keep the drianage bag above the floor, but below the bladder level to prevent reflux / contamination : YES
Fall Risk Assessment Tool

Total Fall Risk Score(sum of all points per category) : 13
Remarks : 13-MODERATE FALL RISK
Age : Below 60 Years - 0
Fall History : One fall within 6 months before admission- 5
Elimination,Bowel and Urine : Incontinence- 2
Patient care Equipment : 3 or more Present - 3
Mobility : Unsteady Gait- 2
Cognition : Altered awareness of immediate physical environment- 1
Type of Assessment : Shift Assessment
Skin Examination



Hospital No: 66415670
Name:

Visit No:
Doctor Name:

Pallor : No
Cyanosis : No
Jaundice : No
Erythema : No
Petechiae : No
Oedema : No
Rash : No
Skin Lesions : No
Surgical scars : No
Wounds : Yes
Location : pressure ulcer present in sacral region
Burns : No
Other : ,(heled) 2nd degree bed sore
Hair : Normal

Nails : Normal

Pain Assessments
Score: 2
Type of Assessment: Shift Assessment
Pain Assessment Scale: Numeric Pain Scale (0-10 Scale)
Numeric Pain Scale: 2

Additional Pain Assessment Details

Since: 25/03/2026
Type: Dull
Duration: Episodic
Continuous: Variable

Comments: 2/10

Intravascular Access Lines
Status : Entered
IntraVascular Access Type : Peripheral IV Line
Peripheral IV Activity : New insertion
Peripheral IV Insertion Date : 24/03/2026
Peripheral IV Insertion Time : 12:00
Peripheral IV Catheter Type : IV Cannula
Peripheral IV Location : Left
Peripheral IV Site : External jugular



Hospital No: 66415670
Name:

Visit No:
Doctor Name:

Peripheral IV Gauge : 16 Gauge
Peripheral IV Procedure Preparation : Hand hygiene performed
Peripheral IV Insertion Indication : IV hydration / medication
Peripheral IV Procedure Result : Successful insertion
Peripheral IV Number Of Attempts : 1
Peripheral IV Equipment used : Extension set
Referred To IV Nurse : No
CVC Site Assesst

Lumen color : grey
Central IV TPA Intervention

Catheter position corrected / withdrawn

Arterial Line Status

Peripheral IV Assessment
Need Assessment :
Yes
Care : Secured with
tape
Site Details : No
complications
Dressing Type / Appearance :
Intact
Peripheral IV Patency : No complications

Patient Acuity Tool
Stable patient 1 to 11
Moderate Risk Patient 12 to 22
Complex Patient 22 to 33
High Risk Patient >34

Total Score :17
ADULT & PAEDIATRIC - ASPIRATION RISK ASSEMENT TOOL
Status : Entered
Score : 0
CATEGORIES
MEDICAL DIAGNOSIS : Other Movement Disorders - 1, Seizure Disorder -
1
EATING HABILTS : On Tube feeding - 3

TOTAL SCORE : 5
COMMENTS : 5-MODERATE RISK
Adult Pressure Ulcer Braden Scale
Status : Entered
SENSORY PERCEPTION : 2



Hospital No: 66415670
Name:

Visit No:
Doctor Name:

2 - Very Limited : Y
MOISTURE : 2
2 - Moist : Y
ACTIVITY : 1
1 - Bed Bound : Y
MOBILITY : 2
2 - Very Limited : Y
NUTRITION : 2
2 - Probably Inadequate : Y
FRICTION AND SHEAR : 1
1 - Problem : Y
TOTAL SCORE : 10-HIGH RISK
Status : Entered
Questionnaires
Status : Entered
Date : 25/03/2026
Patient's Identification No. : MH017532346
Sex : Male
A - Each Risk Factor Represents 1 Point : Age 41 - 60 years, Sepsis (< 1 month), Other risk factor
Total Tickmarks : 3
Total Score A : 3
Total Score (A+B+C+D+E) = : 3
Risk Assessment : : High
3 - 4 High : 3-HIGH RISK

Forms

Add Nursing Care Plan
Status : Entered
Nursing Care Plan discussed with Patient : No
Nursing Care Plan discussed with Family : Yes
Patient Measurable Goal for the day : TO MAINTAIN NORMAL NUTRITION Nursing Diagnosis : Imbalance nutrition less than body requirement due to intolerance of diet.
Nursing Interventions : A) ASSES THE PATIENT NUTRITIONAL STATUS
B) PROVIDED NUTRITIONAL RICH DIET
C) PROVIDED FLUIDS AS ADVISED
D) ADVISED TO TAKE NUTRITION RICH DIET LIKE FRUITS ETC
Evaluation : Continue
Add Nursing Care Plan
Status : Entered
Nursing Care Plan discussed with Patient : No
Nursing Care Plan discussed with Family : Yes
Patient Measurable Goal for the day : TO PREVENT FALL Nursing Diagnosis : Risk for fall
Nursing Interventions : A) PROVIDED COMFORT TO PATIENT
B) ENSURE THE THERAPEUTIC PRESENCE WITH PATIENT TO REDUCE THE FALL RISK
C) PROVIDE SIDE RAILS
D) PROVIDED EDUCATION ON SAFETY MEASURE TO PATIENT AND ATTENDER
Evaluation : Continue
Add Nursing Care Plan



Hospital No: 66415670
Name:

Visit No:
Doctor Name:

Status : Entered
Nursing Care Plan discussed with Patient : No
Nursing Care Plan discussed with Family : Yes
Patient Measurable Goal for the day : TO PREVENT DVT
Nursing Diagnosis : Risk for DVT
Nursing Interventions : A) PROVIDED COMFORTABLE POSITION
B) ASSES THE CONDITION OF PATIENT
C) ENCOURAGE THE REGULAR EXERCISE AND LIMB ELEVATION, MOBILITY
D) APPLY MECHANICAL AND PHARMACOLOGICAL DVT MEASURES

Evaluation : Continue
Patient/Family Health Education
Status : Entered
Date : 25/03/2026
Time : 15:05
Assessment Type : Other1

Educational Needs : Risk,Safety,Restrain precaution,Hand hygiene
Given Education : GIVEN
Teaching Method : One to One
Teaching Tool : Verbal Instructions
Child Attends School : Not Applicable
Does the child has any problem at school : Not Applicable

Progress Note :
LP NOTES:
LP done with all aseptic precautions using 22G needle and clear CSF drained ~ 2-3ml / sample x 3 samples. patient tolerated the procedure well. Send CSF for protein, sugar, cell count and cell type.
Corresponding GRBS - 166mg/dl

Investigation Orders

Laboratory : - CSF (Protein , Sugar, Chloride)
- CSF Cell Count and Cell Type(E)
Procedure : - LUMBAR PUNCTURE

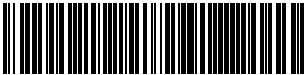
MEDICATION ORDERS

Drug / Generic Item	Dosage Qty	Frequency	Instructions/Notes	Duration
MEDIPOT 1.5GM/10ML INJECTION (POTASSIUM CHLORIDE (80008163))	2 AMP	ONCE		1 DAY(S)
NORMAL SALINE 0.9% W/V 100 ML AQUA PULSE, DENIS (SODIUM CHLORIDE (80009680))	4 BT	ONCE		1 DAY(S)
MAGNESIUM SULPHATE 50% W/V 2ML AMP SOL FOR INJ (MAGNESIUM SULFATE (80006731))	4 AMP	ONCE		1 DAY(S)
PANTOCID 40 MG/VIAL LYOPHILIZED POWDER FOR INJECTION (PANTOPRAZOLE (80007606))	2 VL	MORNING DOSE(OD (1-0-0))		1 DAY(S)
MAGNEX FORTE 1.5 GM/VIAL POWDER FOR INJECTION (CEFOPERAZONE~SULBACTAM (80002516))	1 VL	BD (1-0-1)		1 DAY(S)
NORMAL SALINE 0.9% W/V AQUA PULSE 500 ML (SODIUM CHLORIDE (80009680))	5 BT	ONCE		1 DAY(S)
NORMAL SALINE 0.9% W/V 100 ML AQUA PULSE, DENIS (SODIUM CHLORIDE (80009680))	8 BT	ONCE		1 DAY(S)



Hospital No: 66415670	Visit No:
Name:	Doctor Name:

AUTOFUSION SET # 14289 (IV SET (PHARMACY NON-DRUG))	5 PC	ONCE		1 DAY(S)
SYRINGE 10 ML WITH NEEDLE DISPOVAN, HMD (SYRINGE (PHARMACY NON-DRUG))	10 PC	ONCE		1 DAY(S)
BEPARIN 5000 IU/5 ML VIAL SOLUTION FOR INJECTION, 1000 I.U./ML (HEPARIN (80005278))	1 VL	ONCE		1 DAY(S)
DIAPERS ADULT (XL)UNISEX B-EAZY 1'S, NUSAAC MEDIQUIPTS (SURGICALS (NON PHARMA DRUG))	5 PC	ONCE		1 DAY(S)
UNDERPAD ABSORABLE MEDICAL(60 X90CM) ULTRACARE (UNDERPAD (PHARMACY NON-DRUG))	5 NUMBER	ONCE		1 DAY(S)
BED BATH TOWEL STERIX 320X320, BAPUJI (SURGICAL)	3 PC	ONCE		1 DAY(S)
R-CIN 450 MG CAPSULE (RIFAMPICIN (80009282))	1 CAP	MORNING DOSE(OD (1-0-0))		2 DAY(S)
SOLONEX 300 MG TABLET (ISONIAZID (80005819))	1 TAB	MORNING DOSE(OD (1-0-0))		2 DAY(S)
COMBUTOL 800 MG TABLET (ETHAMBUTOL (80004239))	1 TAB	MORNING DOSE(OD (1-0-0))		2 DAY(S)
BENADON 40 MG TABLET 15'S (PYRIDOXINE), BAYER (PYRIDOXINE (80008711))	1 TAB	ONCE		1 DAY(S)
SHELCAL 500 MG TABLET (CALCIUM CARBONATE~VITAMIN D (80001842))	1 TAB	MORNING DOSE(OD (1-0-0))		2 DAY(S)
LEVACETAM 500MG INJECTION 5ML (LEVETIRACETAM (80006107))	1 ML	TID. (1-1-1)		1 DAY(S)
DIGITAL THERMOMETER, MYCARE (APPLIANCE (PHARMACY NON-DRUG))	1 NUMBER	ONCE		1 DAY(S)
WATER FOR INJECTION STERILE GLASS BOTTLE 500 ML	1 BT	ONCE		1 DAY(S)
LOX 2% W/V JELLY 30 GM (LIDOCAINE (80006251))	1 PC	ONCE		1 DAY(S)
CANNULA 16 G VENFLON (1.7X45MM) INSTAFLASH # 391595, BECTON DICKINSON (CANNULA (PHARMACY NON-DRUG))	1 PC	ONCE		1 DAY(S)
TEGADERM IV DRESSING KIT 6.5 CM X 7 CM, # 1683 (TEGADERM DRESSING (PHARMACY NON-DRUG))	1 PC	ONCE		1 DAY(S)
HUMAN ACTRAPID 40 IU/ML 10 ML VIAL SOLUTION FOR INJECTION (INSULIN HUMAN (80005489))	1 VL	ONCE		1 DAY(S)
SYRINGE 1 CC INSULIN 40 IU DISPOVAN, HMD (SYRINGE (PHARMACY NON-DRUG))	5 PC	ONCE		1 DAY(S)
LEVACETAM 500MG INJECTION 5ML (LEVETIRACETAM (80006107))	2 SOLUTION FOR INJECTIONS	EVERY 8 HRS(8 HOURLY (Q8H))		1 DAY(S)
PANTOCID 40 MG/VIAL LYOPHILIZED POWDER FOR INJECTION (PANTOPRAZOLE (80007606))	1 VL	ONCE		1 DAY(S)
VIATRAN 1.5 GM/VIAL POWDER FOR INJECTION (CEFOPERAZONE~SULBACTAM (80002516))	1 VL	12 HOURLY (Q12H)		1 DAY(S)
LEVACETAM 500MG INJECTION 5ML (LEVETIRACETAM (80006107))	2 SOLUTION FOR INJECTIONS	EVERY 8 HRS(8 HOURLY (Q8H))		1 DAY(S)
NORMAL SALINE 09% W/V AQUA PULSE 500 ML	2 BT	ONCE		1 DAY(S)



Hospital No: 66415670

Visit No:

Name:

Doctor Name:

(SODIUM CHLORIDE (80009680))				
NORMAL SALINE 0.9% W/V 100 ML AQUA PULSE, DENIS	5 BT	ONCE		1 DAY(S)
(SODIUM CHLORIDE (80009680))				
SYRINGE 10 ML WITH NEEDLE DISPOVAN, HMD	8 PC	ONCE		1 DAY(S)
(SYRINGE (PHARMACY NON-DRUG))				
SYRINGE 2 ML WITH NEEDLE DISPOVAN, HMD)	5 PC	ONCE		1 DAY(S)
(SYRINGE (PHARMACY NON-DRUG))				
R-CIN 450 MG CAPSULE	1 CAP	ONCE		5 DAY(S)
(RIFAMPICIN (80009282))				
SOLONEX 300 MG TABLET	1 TAB	ONCE		5 DAY(S)
(ISONIAZID (80005819))				
COMBUTOL 800 MG TABLET	1 TAB	ONCE		2 DAY(S)
(ETHAMBUTOL (80004239))				
SHELCAL 500 MG TABLET	1 TAB	ONCE		4 DAY(S)
(CALCIUM CARBONATE~VITAMIN D (80001842))				
50CC DISPOVAN SYRINGE WITHOUT NEEDLE LUER MOUNT	2 NUMBER	ONCE		1 DAY(S)
(SYRINGE (PHARMACY NON-DRUG))				
NEEDLE DISPOSABLE 18G X 1.1/2, DISPOVAN	2 PC	ONCE		1 DAY(S)
(NEEDLE (PHARMACY NON-DRUG))				
SYRINGE 5 ML WITH NEEDLE DISPOVAN, HMD	5 PC	ONCE		1 DAY(S)
(SYRINGE (PHARMACY NON-DRUG))				
LACOSAM INJECTION 20ML	2 PC	ONCE		1 DAY(S)
(LACOSAMIDE (80005919))				
SYRINGE 2 ML WITH NEEDLE DISPOVAN, HMD)	5 PC	ONCE		1 DAY(S)
(SYRINGE (PHARMACY NON-DRUG))				